centre for pediatric emergencies and specialized care

INFORMATION SHEET ON ORAL IMMUNOTHERAPY (OIT)

How can OIT help my food allergies?

Allergies are caused by an error of the immune system; it sees the allergens (ex: peanuts) as a threat and declares war even though the allergen itself is not harmful. The goal of OIT is to show the immune system that the allergen is NOT a threat, and that it doesn't have to react every time it sees the food. By doing so, OIT can reduce the risk of severe allergic reaction (ex: anaphylaxis) when you are exposed to the allergens.

OIT is a long process that requires close follow-up. The process requires to eat a small quantity (the "dose") of the food every day, and to slowly increase the dose under medical supervision until the ideal dose is achieved.

How often are the visits with my allergist?

OIT can be divided into two phases:

1. **Escalation phase** – appointments with your allergist are every 6-8 weeks to increase the dose, during which you will be observed in the clinic under for 60 minutes. Between visits, you need to take the dose every day at home under parental supervision. This phase usually takes 12-18 months.
2. **Maintenance phase** – once the ideal dose is achieved, this dose will need to be taken regularly (minimum 3 times per week) at home albeit visits with your allergist will be once every 3 to 12 months.

Can OIT cure my food allergy?

No, OIT cannot cure food allergies; the goal is to help you tolerate a big enough quantity of the food so that in case of accidental exposure, you would be protected against severe reactions. Approximately 80% of children reach this state of protection. When achieved, you need to continue eating the food regularly to maintain this level protection. If you stop taking this dose for several weeks/months, 3 out of 4 patients lose the protection achieved.

Are there risks or side effect with OIT?

Yes, allergic reactions can occur, even with small doses. Most often, reactions happen during the escalation phase and are mild (hives, abdominal pain, throat itching, etc.), but can also be worse (vomiting, asthma attack, etc.) and in rare cases, anaphylaxis. Mild reactions should be treated with non-sedating antihistamine such as cetirizine (Reactine); severe reactions should be treated with epinephrine autoinjector such as Epipen. After injection of epinephrine, one must absolutely go to the emergency room.

It is important to discuss with your allergist if any new symptoms develop during OIT.

Do we need a referral to talk about OIT with an allergist?

Yes, a referral from a doctor is required. A clear PDF document must be uploaded with your request for appointment in the allergy program.